

SENATE No. 3069

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

SENATE, May 18, 2026.

The committee on Health Care Financing to whom was referred the Senate Bill ensuring access to healthcare and medically necessary food for children (Senate, No. 691), - reports, recommending that the same ought to pass with an amendment substituting a new draft with the same title (Senate, No. 3069) (estimated cost greater than \$100,000).

For the committee,
Cindy F. Friedman

SENATE No. 3069

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In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act ensuring access to healthcare and medically necessary food for children.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 32A of the General Laws is hereby amended by inserting after
2 section 34 the following section:-

3 Section 35. (a) For the purpose of this section, the following words shall have the
4 following meanings:

5 “Food protein allergy”, Immunoglobulin E and non-Immunoglobulin E-mediated
6 allergies to food proteins, including: (i) food protein-induced enterocolitis syndrome; (ii)
7 Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins; and (iii)
8 Eosinophilic disorders, including eosinophilic esophagitis, eosinophilic gastroenteritis,
9 eosinophilic colitis, and post-transplant eosinophilic disorders.

10 “Medically necessary food”, a low protein modified food product, an amino acid
11 preparation product, a modified fat preparation product, or a nutritional formula, including such a
12 formula that does not require a prescription, that is: (i) furnished pursuant to the prescription,
13 order, or recommendation, as applicable, of a physician or other health care professional

qualified to make such prescription, order, or recommendation, for the dietary management of a food protein allergy; (ii) a specially formulated and processed product for the partial or exclusive feeding of an individual by means of oral intake or enteral feeding by tube; (iii) intended for the dietary management of an individual who, because of therapeutic or chronic medical needs, has limited or impaired capacity to ingest, digest, absorb, or metabolize ordinary food stuffs or certain nutrients, or who has other special medically determined nutrient requirements, the dietary management of which cannot be achieved by the modification of the normal diet alone; (iv) intended to be used under medical supervision, which may include in a home setting; and (v) intended only for an individual receiving active and ongoing medical supervision wherein the individual requires medical care on a recurring basis for, among other things, instructions on the use of the food. The term “Medically necessary food” shall not include: (i) foods taken as part of an overall diet designed to reduce the risk of a disease or medical condition or as weight loss products, even if they are recommended by a physician or other health professional; (ii) foods marketed as gluten-free for the management of celiac disease or non-celiac gluten sensitivity; and (iii) foods marketed for the management of diabetes.

(b) The commission shall provide coverage for the cost of medically necessary food from a supplier participating in such carrier’s health provider network, for persons with a food protein allergy under the age of 18. An increase in patient cost sharing shall not be allowed to achieve compliance with this section.

(c) The commission shall not: (i) require an insured to obtain a referral from a primary care provider for specialty care, provided by an immunologist or family practitioner participating in such carrier's health care provider network, related to medically necessary food for a food protein allergy for persons under the age of 18 as described in this section; or (ii) categorize

prescription coverage for medically necessary food for a food protein allergy for persons under the age of 18 as “durable medical equipment.”

SECTION 2. Chapter 175 of the General Laws is hereby amended by inserting after section 230 the following section:-

Section 231. (a) For the purpose of this section, the following words shall have the following meanings:

“Food protein allergy”, Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins, including: (i) food protein-induced enterocolitis syndrome; (ii) Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins; and (iii) Eosinophilic disorders, including eosinophilic esophagitis, eosinophilic gastroenteritis, eosinophilic colitis, and post-transplant eosinophilic disorders.

“Medically necessary food”, a low protein modified food product, an amino acid preparation product, a modified fat preparation product, or a nutritional formula, including such a formula that does not require a prescription, that is: (i) furnished pursuant to the prescription, order, or recommendation, as applicable, of a physician or other health care professional qualified to make such prescription, order, or recommendation, for the dietary management of a food protein allergy; (ii) a specially formulated and processed product for the partial or exclusive feeding of an individual by means of oral intake or enteral feeding by tube; (iii) intended for the dietary management of an individual who, because of therapeutic or chronic medical needs, has limited or impaired capacity to ingest, digest, absorb, or metabolize ordinary food stuffs or certain nutrients, or who has other special medically determined nutrient requirements, the dietary management of which cannot be achieved by the modification of the normal diet alone;

(iv) intended to be used under medical supervision, which may include in a home setting; and (v) intended only for an individual receiving active and ongoing medical supervision wherein the individual requires medical care on a recurring basis for, among other things, instructions on the use of the food. The term “Medically necessary food” shall not include: (i) foods taken as part of an overall diet designed to reduce the risk of a disease or medical condition or as weight loss products, even if they are recommended by a physician or other health professional; (ii) foods marketed as gluten-free for the management of celiac disease or non-celiac gluten sensitivity; and (iii) foods marketed for the management of diabetes.

(b) Any policy, contract, agreement, plan or certificate of insurance issued, delivered or renewed within the commonwealth, which is considered creditable coverage under section 1 of chapter 111M, shall provide coverage for the cost of medically necessary food from a supplier participating in such carrier’s health provider network, for persons with a food protein allergy under the age of 18. An increase in patient cost sharing shall not be allowed to achieve compliance with this section.

(c) A carrier under this chapter, as defined in section 1 of chapter 12C, shall not: (i) require an insured to obtain a referral from a primary care provider for specialty care, provided by an immunologist or family practitioner participating in such carrier's health care provider network, related to medically necessary food for a food protein allergy for persons under the age of 18 as described in this section; or (ii) categorize prescription coverage for medically necessary food for a food protein allergy for persons under the age of 18 as “durable medical equipment.”

SECTION 3. Chapter 176A of the General Laws is hereby amended by inserting after section 38 the following section:-

Section 39. (a) For the purpose of this section, the following words shall have the following meanings:

“Food protein allergy”, Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins, including: (i) food protein-induced enterocolitis syndrome; (ii) Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins; and (iii) Eosinophilic disorders, including eosinophilic esophagitis, eosinophilic gastroenteritis, eosinophilic colitis, and post-transplant eosinophilic disorders.

“Medically necessary food”, a low protein modified food product, an amino acid preparation product, a modified fat preparation product, or a nutritional formula, including such a formula that does not require a prescription, that is: (i) furnished pursuant to the prescription, order, or recommendation, as applicable, of a physician or other health care professional qualified to make such prescription, order, or recommendation, for the dietary management of a food protein allergy; (ii) a specially formulated and processed product for the partial or exclusive feeding of an individual by means of oral intake or enteral feeding by tube; (iii) intended for the dietary management of an individual who, because of therapeutic or chronic medical needs, has limited or impaired capacity to ingest, digest, absorb, or metabolize ordinary food stuffs or certain nutrients, or who has other special medically determined nutrient requirements, the dietary management of which cannot be achieved by the modification of the normal diet alone; (iv) intended to be used under medical supervision, which may include in a home setting; and (v) intended only for an individual receiving active and ongoing medical supervision wherein the individual requires medical care on a recurring basis for, among other things, instructions on the use of the food. The term “Medically necessary food” shall not include: (i) foods taken as part of an overall diet designed to reduce the risk of a disease or medical condition or as weight loss

products, even if they are recommended by a physician or other health professional; (ii) foods marketed as gluten-free for the management of celiac disease or non-celiac gluten sensitivity; and (iii) foods marketed for the management of diabetes.

(b) Any contract between a subscriber and the corporation under an individual or group hospital service plan that is delivered, issued or renewed within the commonwealth shall provide coverage for the cost of medically necessary food from a supplier participating in such carrier's health provider network, for persons with a food protein allergy under the age of 18. An increase in patient cost sharing shall not be allowed to achieve compliance with this section.

(c) A carrier under this chapter, as defined in section 1 of chapter 12C, shall not: (i) require an insured to obtain a referral from a primary care provider for specialty care, provided by an immunologist or family practitioner participating in such carrier's health care provider network, related to medically necessary food for a food protein allergy for persons under the age of 18 as described in this section; or (ii) categorize prescription coverage for medically necessary food for a food protein allergy for persons under the age of 18 as "durable medical equipment."

SECTION 4. Chapter 176B of the General Laws is hereby amended by inserting after section 25 the following section:-

Section 26. (a) For the purpose of this section, the following words shall have the following meanings:

"Food protein allergy", Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins, including: (i) food protein-induced enterocolitis syndrome; (ii) Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins; and (iii)

Eosinophilic disorders, including eosinophilic esophagitis, eosinophilic gastroenteritis, eosinophilic colitis, and post-transplant eosinophilic disorders.

“Medically necessary food”, a low protein modified food product, an amino acid preparation product, a modified fat preparation product, or a nutritional formula, including such a formula that does not require a prescription, that is: (i) furnished pursuant to the prescription, order, or recommendation, as applicable, of a physician or other health care professional qualified to make such prescription, order, or recommendation, for the dietary management of a food protein allergy; (ii) a specially formulated and processed product for the partial or exclusive feeding of an individual by means of oral intake or enteral feeding by tube; (iii) intended for the dietary management of an individual who, because of therapeutic or chronic medical needs, has limited or impaired capacity to ingest, digest, absorb, or metabolize ordinary food stuffs or certain nutrients, or who has other special medically determined nutrient requirements, the dietary management of which cannot be achieved by the modification of the normal diet alone; (iv) intended to be used under medical supervision, which may include in a home setting; and (v) intended only for an individual receiving active and ongoing medical supervision wherein the individual requires medical care on a recurring basis for, among other things, instructions on the use of the food. The term “Medically necessary food” shall not include: (i) foods taken as part of an overall diet designed to reduce the risk of a disease or medical condition or as weight loss products, even if they are recommended by a physician or other health professional; (ii) foods marketed as gluten-free for the management of celiac disease or non-celiac gluten sensitivity; and (iii) foods marketed for the management of diabetes.

(b) Any subscription certificate under an individual or group medical service agreement delivered, issued or renewed by agreement within the commonwealth shall provide coverage for

the cost of medically necessary food from a supplier participating in such carrier's health provider network, for persons with a food protein allergy under the age of 18. An increase in patient cost sharing shall not be allowed to achieve compliance with this section.

(c) A carrier under this chapter, as defined in section 1 of chapter 12C, shall not: (i) require an insured to obtain a referral from a primary care provider for specialty care, provided by an immunologist or family practitioner participating in such carrier's health care provider network, related to medically necessary food for a food protein allergy for persons under the age of 18 as described in this section; or (ii) categorize prescription coverage for medically necessary food for a food protein allergy for persons under the age of 18 as "durable medical equipment."

SECTION 5. Chapter 176G of the General Laws is hereby amended by inserting after section 33 the following section:-

Section 34. (a) For the purpose of this section, the following words shall have the following meanings:

"Food protein allergy", Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins, including: (i) food protein-induced enterocolitis syndrome; (ii) Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins; and (iii) Eosinophilic disorders, including eosinophilic esophagitis, eosinophilic gastroenteritis, eosinophilic colitis, and post-transplant eosinophilic disorders.

"Medically necessary food", a low protein modified food product, an amino acid preparation product, a modified fat preparation product, or a nutritional formula, including such a formula that does not require a prescription, that is: (i) furnished pursuant to the prescription, order, or recommendation, as applicable, of a physician or other health care professional

qualified to make such prescription, order, or recommendation, for the dietary management of a food protein allergy; (ii) a specially formulated and processed product for the partial or exclusive feeding of an individual by means of oral intake or enteral feeding by tube; (iii) intended for the dietary management of an individual who, because of therapeutic or chronic medical needs, has limited or impaired capacity to ingest, digest, absorb, or metabolize ordinary food stuffs or certain nutrients, or who has other special medically determined nutrient requirements, the dietary management of which cannot be achieved by the modification of the normal diet alone; (iv) intended to be used under medical supervision, which may include in a home setting; and (v) intended only for an individual receiving active and ongoing medical supervision wherein the individual requires medical care on a recurring basis for, among other things, instructions on the use of the food. The term “Medically necessary food” shall not include: (i) foods taken as part of an overall diet designed to reduce the risk of a disease or medical condition or as weight loss products, even if they are recommended by a physician or other health professional; (ii) foods marketed as gluten-free for the management of celiac disease or non-celiac gluten sensitivity; and (iii) foods marketed for the management of diabetes.

(b) An individual group health maintenance contract that is issued or renewed within or without the commonwealth shall provide coverage for the cost of medically necessary food from a supplier participating in such carrier’s health provider network, for persons with a food protein allergy under the age of 18. An increase in patient cost sharing shall not be allowed to achieve compliance with this section.

(c) A carrier under this chapter, as defined in section 1 of chapter 12C, shall not: (i) require an insured to obtain a referral from a primary care provider for specialty care, provided by an immunologist or family practitioner participating in such carrier's health care provider

network, related to medically necessary food for a food protein allergy for persons under the age of 18 as described in this section; or (ii) categorize prescription coverage for medically necessary food for a food protein allergy for persons under the age of 18 as “durable medical equipment.”

SECTION 6. Chapter 176I of the General Laws is hereby amended by inserting after section 13 the following section:-

Section 14. (a) For the purpose of this section, the following words shall have the following meanings:

“Food protein allergy”, Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins, including: (i) food protein-induced enterocolitis syndrome; (ii) Immunoglobulin E and non-Immunoglobulin E-mediated allergies to food proteins; and (iii) Eosinophilic disorders, including eosinophilic esophagitis, eosinophilic gastroenteritis, eosinophilic colitis, and post-transplant eosinophilic disorders.

“Medically necessary food”, a low protein modified food product, an amino acid preparation product, a modified fat preparation product, or a nutritional formula, including such a formula that does not require a prescription, that is: (i) furnished pursuant to the prescription, order, or recommendation, as applicable, of a physician or other health care professional qualified to make such prescription, order, or recommendation, for the dietary management of a food protein allergy; (ii) a specially formulated and processed product for the partial or exclusive feeding of an individual by means of oral intake or enteral feeding by tube; (iii) intended for the dietary management of an individual who, because of therapeutic or chronic medical needs, has limited or impaired capacity to ingest, digest, absorb, or metabolize ordinary food stuffs or certain nutrients, or who has other special medically determined nutrient requirements, the

dietary management of which cannot be achieved by the modification of the normal diet alone;
(iv) intended to be used under medical supervision, which may include in a home setting; and (v)
intended only for an individual receiving active and ongoing medical supervision wherein the
individual requires medical care on a recurring basis for, among other things, instructions on the
use of the food. The term “Medically necessary food” shall not include: (i) foods taken as part of
an overall diet designed to reduce the risk of a disease or medical condition or as weight loss
products, even if they are recommended by a physician or other health professional; (ii) foods
marketed as gluten-free for the management of celiac disease or non-celiac gluten sensitivity;
and (iii) foods marketed for the management of diabetes.

(b) A preferred provider contract between a covered person and an organization shall
provide coverage for the cost of medically necessary food from a supplier participating in such
carrier’s health provider network, for persons with a food protein allergy under the age of 18. An
increase in patient cost sharing shall not be allowed to achieve compliance with this section.

(c) A carrier under this chapter, as defined in section 1 of chapter 12C, shall not: (i)
require an insured to obtain a referral from a primary care provider for specialty care, provided
by an immunologist or family practitioner participating in such carrier's health care provider
network, related to medically necessary food for a food protein allergy for persons under the age
of 18 as described in this section; or (ii) categorize prescription coverage for medically necessary
food for a food protein allergy for persons under the age of 18 as “durable medical equipment.”

SECTION 7. Not later than 6 months after the enactment of this act, the division of
insurance shall promulgate the rules and regulations necessary for the implementation and
enforcement of the sections of this act.